



PRITAREN TABLET 50MG

Description: Light brown, round, biconvex, top-scored tablet.
Enteric-coated.

Content:
Each tablet contains diclofenac sodium 50mg

Indication:
For treatment of inflammatory and degenerative forms of rheumatism, rheumatoid arthritis, osteoarthritis, ankylosing spondylitis and acute gout.

Pharmacology :

Diclofenac, a phenylacetic acid derivative is a non-steroidal antiinflammatory agent which has analgesic, antipyretic and antiinflamm properties. It is an inhibitor of cyclooxygenase of the prostaglandin biosynthesis. Diclofenac is used mainly as the sodium salt for the relief of and inflammation conditions such as rheumatoid arthritis, osteoarthritis, ankylosing spondylitis, renal colic, acute gout and following some surgical procedures. In more acute inflammatory diseases, diclofenac has an effective antiinflammatory and analgesic action. It has proven effect the treatment of acute gout and it is also permissible in the treatment of juvenile chronic arthritis. It was found that diclofenac is effective relieving pain and stiffness and more effective in improving hip- or knee - joint movement in patients with osteoarthritis of the hip or knees. Diclofenac was detected in synovial fluid 2 hours after a dose and the concentration remained relatively constant for the next 9 hours. Therapeutic concentrations, it is more than 99% bound to plasma proteins. Diclofenac is metabolised to 4'-hydroxydiclofenac, 5-hydroxydiclofenac, 3'-hydroxydiclofenac and 4'-, 5'-dihydroxydiclofenac. It is excreted in the form of glucuronide and sulphate conjug mainly in the urine. It is also excreted through the bile.

Adverse effects :

The most frequently reported adverse effects are gastrointestinal effects. Adverse effects associated with diclofenac are mild and transient and appear to be unrelated to the dose of the drug given. Rectal administration of diclofenac suppositories cause local reactions such as itching, burning or exacerbation of haemorrhoids. **Gastrointestinal:** epigastric pain, nausea, vomiting and diarrhoea. Peptic ulcer and GI bleeding rarely occurs. **Kidneys:** renal papillary necrosis and nephrotic syndrome may occur. **Liver:** elevations of serum aminotransferase activity and clinical hepatitis. **Blood:** aplastic anaemia, hemolytic anaemia, thrombocytopenia and agranulocytosis. Localised spontaneous bleeding, bruising, inhibition of platelet aggregation and prolonged bleeding time. **Central nervous system:** headache, dizziness, tinnitus, insomnia, depression and drowsiness. **Special senses:** disturbance of the sensory, memory, vision, taste and tremor. **Skin:** occasionally rashes or skin eruptions may occur. Cases of hair bulbous eruptions, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Leyell's syndrome) photosensitivity reactions have been reported. **Cardiovascular:** isolated cases of palpitation, chest pains and hypertension.

Cardiac disorders :

Unknown*: Myocardial infarction, cardiac failure, palpitations, chest pain.
*The frequency reflects data from long term treatment with a high dose (150 mg/day).

Description of selected adverse drug reactions

Arteriothrombotic events

Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150 mg daily) and during long-term treatment (see section WARNINGS AND PRECAUTIONS).

Warnings and Precautions

Cardiovascular effects

Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, maybe associated with an increased serious cardiovascular thrombotic events (including myocardial infarction and stroke).

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at 100mg daily when treatment continues for more than 4 weeks.

As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.

Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. : chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings. Patients should be instructed to see a physician immediately if any of such an event.

Patients receiving long term treatment with diclofenac should have periodic-blood counts. It should be given with care to elderly, to patients with asthma or bronchospasm, bleeding disorders, cardiovascular diseases, a history of peptic ulcer and in liver or renal failure. Care is also required in those patients who are also receiving coumarin anticoagulant aspirin-sensitive patients. Severe cutaneous reactions, including Stevens-Johnson syndrome and toxic epidermal necrolysis (Leyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs. Like other NSAIDs may mask signs and symptoms of infection due to its pharmacodynamic properties.

Cardiovascular Thrombotic Events

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors lowest effective dose should be used for the shortest possible duration.

There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAIDs treatment and at regular intervals thereafter.



Heart Failure

Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients, receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Drug Interactions:

Triamterene, cyclosporin - deterioration of renal function in concomitant use with diclofenac.
Digoxin, lithium and methotrexate - Increased in blood concentration is observed.
Salicylates - Pharmacokinetic interactions when co-administered with diclofenac in anti-inflammatory doses.
Quinolone antibiotics - Increased in central nervous system effects leading to convulsion.
Other NSAIDs and steroids - Increased in frequency of adverse reactions.
Anticoagulants - Increased risk of haemorrhage.

Contraindications:

It is contraindicated in patients hypersensitive to diclofenac; active or suspected gastrointestinal ulcers or bleeding, patients in whom attacks of asthma, urticaria or acute rhinitis are precipitated by aspirin or other NSAIDs; and, in ulcerative or acute inflammatory conditions of the anus, rectum and sigmoid colon.

* Severe cardiac failure (see section WARNINGS AND PRECAUTIONS).

Dosage and administration:

Adults: Oral, 25-50mg three times daily, or as directed. Children: As directed by physician.
After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

As a general recommendation, the dose should be individually adjusted.

Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms (see section WARNINGS AND PRECAUTIONS).

Established cardiovascular disease or significant cardiovascular risk factors

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100 mg daily if treated for more than 4 weeks (see section WARNINGS AND PRECAUTIONS).

Treatment of overdose:

Management of overdose is usually symptomatic and supportive. Owing to the high protein binding rate and extensive metabolism, forced diuresis, dialysis or hemoperfusion are not helpful.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Storage conditions : Keep container tightly closed. Store in a cool, dry place below 30°C. Protect from light. Keep away from children.

Shelf life : 3 years

Presentation : 100 x 10's/blister

Manufactured & Distributed by : PRIME PHARMACEUTICAL SDN. BHD.

1505, Lrg Perusahaan Utama 1, Taman Perindustrian Bukit Tengah, 14000 Bukit Mertajam, Penang Malaysia.